

Medicare Part D — Prescription Drug Plan

The Field Guide Nobody Handed You

25 years of U.S. Medicare field wisdom on Part D — what agents get wrong, what clients don't know, and what the rulebook never tells you until it's too late.

Doc: 01 | PDP Brief | U.S. Medicare Agents | AEP 2026 | **Brand:** AgentRisePro / 121 Urban Media LLC | **Audience:** U.S. Medicare Agents & Beneficiaries Only

■ KILL SWITCH — VERIFY BEFORE EVERY USE

- AEP 2026 dates: October 15 – December 7, 2025. Changes effective January 1, 2026. Verify at [cms.gov](https://www.cms.gov).
- Part D national base beneficiary premium changes annually. Verify at [cms.gov](https://www.cms.gov) before calculating LEP.
- Extra Help / LIS income thresholds change annually. Always verify at [ssa.gov](https://www.ssa.gov).
- Part D Coverage Gap (Donut Hole) eliminated in 2025 under the Inflation Reduction Act. Confirm no future legislative reversal.
- Plan formularies change every plan year. Never assume last year's drug tier is the same this year.
- TrOOP threshold for catastrophic coverage: verify current year at [cms.gov](https://www.cms.gov).

"Nobody thinks about their prescriptions until the pharmacy hands them a bill they can't pay. That's when they call you — and by then, the enrollment window may be closed. Your job is to find them before that moment."

WHAT PART D ACTUALLY IS — IN PLAIN LANGUAGE

Medicare Part D is the federal prescription drug benefit for U.S. Medicare beneficiaries. It is not automatic — clients must choose it, enroll, and pay for it separately unless it's bundled into a Medicare Advantage plan. Private insurance companies approved by CMS administer these plans. Every plan has a different formulary, pharmacy network, and cost structure. Your job is to match the client's actual medications to the right plan — not just find the lowest premium.

- **SEASONED AGENT TIP:** Always run a formulary check BEFORE recommending a plan. A \$0 premium plan that excludes a client's \$400 maintenance drug is not a good deal — it's a liability. Use the Medicare Plan Finder at [medicare.gov/plan-compare](https://www.medicare.gov/plan-compare).

U.S. MEDICARE PART D ENROLLMENT WINDOWS

Window	Dates	Who It's For	Agent Action
Initial Enrollment Period (IEP)	7 months: 3 before birthday month, birthday month, 3 after	U.S. beneficiaries new to Medicare at 65	Enroll in Part D or confirm creditable coverage. Miss this and they pay forever.
Annual Enrollment Period (AEP)	Oct 15 – Dec 7 annually. Effective Jan 1.	All Medicare beneficiaries	Your primary selling season. Review every client's plan every year.
Special Enrollment Period (SEP)	Triggered by qualifying life event	Lost creditable coverage, moved, LIS change	Act within the window — SEPs are time-limited by CMS rules.
MA Open Enrollment (OEP)	Jan 1 – Mar 31	MA-PD enrollees only	Can switch MA plans or return to Original Medicare + standalone PDP.

THE LATE ENROLLMENT PENALTY — THE MOST EXPENSIVE MISTAKE IN MEDICARE

If a U.S. Medicare beneficiary goes 63 or more consecutive days without creditable drug coverage after their IEP ends, CMS adds a Late Enrollment Penalty to their Part D premium — permanently. The formula: 1% of the CMS national base beneficiary premium × number of uncovered months. There is no ceiling. It follows them for life.

■ **FIELD WARNING: A client who went 36 months without drug coverage pays a 36% permanent surcharge on their Part D premium. Every month. For the rest of their Medicare life. This is not a scare tactic. It is CMS policy. Educate every client at every appointment.**

"I've seen clients in tears over this. They were healthy, thought they didn't need drug coverage, and nobody warned them. Now you're their warning. Give it to them straight."

THE STAGES OF PART D COVERAGE (2026)

Stage	How It Works	What the Client Pays
1 — Deductible	Client pays 100% of drug costs until annual deductible is met. CMS max was \$590 in 2025. Kill Switch: verify 2026 amount at cms.gov.	100% out of pocket until deductible met
2 — Initial Coverage	Plan and client share drug costs based on tier placement in the formulary.	Copay or coinsurance per drug tier
3 — Catastrophic Coverage	Kicks in after the True Out-of-Pocket (TrOOP) threshold is reached. TrOOP was \$2,000 in 2025. Kill Switch: verify 2026.	Dramatically reduced. \$35/month insulin cap. \$0 for many generics under the Inflation Reduction Act.

Coverage Gap (Donut Hole)	ELIMINATED effective January 1, 2025 under the Inflation Reduction Act. This is a major benefit to communicate to every client.	No longer applies.
---------------------------	---	--------------------

Kill Switch: All Part D thresholds are set annually by CMS. Verify at [cms.gov](https://www.cms.gov) before every client conversation.

WHAT U.S. MEDICARE AGENTS GET WRONG ABOUT PART D

Leading with premium instead of formulary

The cheapest premium plan in your client's zip code is useless if it doesn't cover their medications. Pull up the Medicare Plan Finder. Enter their actual drugs. Show them total annual cost — not just monthly premium. That's the number that matters.

Not screening every client for Extra Help (LIS)

The Low Income Subsidy program can eliminate Part D premiums entirely and reduce cost-sharing to near zero for qualifying beneficiaries. Millions of Americans who qualify never apply because nobody told them. Screen every client. Every time. It takes five minutes and can save them thousands.

Telling VA veterans they're covered for Part D

VA drug benefits are NOT creditable coverage for Medicare Part D purposes. A veteran who relies solely on VA coverage and skips Part D enrollment will owe a Late Enrollment Penalty if they ever want a Part D plan later. Address this at every veteran appointment.

Skiping the pharmacy network check

A plan may cover a drug at a great price — at a preferred pharmacy. At a standard pharmacy, the same drug costs more. Know your client's pharmacy before you recommend a plan. This is a detail that makes or breaks the relationship.

Not reviewing Part D plans every AEP

Formularies, premiums, and pharmacy networks all change on January 1. A plan that was perfect last year may be a bad fit this year. Call every Part D client during AEP, every year, no exceptions. That call is also your referral opportunity.

"Your value as a U.S. Medicare agent is not finding the cheapest premium. It's finding the plan that costs your client the least when they actually use it. That's the difference between an order-taker and an advisor."

VERIFIED U.S. RESOURCES & CITATIONS

Medicare Plan Finder (CMS)

<https://www.medicare.gov/plan-compare/>

Enter client medications and zip code to find lowest total drug cost plan. Use at every AEP appointment.

CMS Part D Benefit Parameters

<https://www.cms.gov/medicare/prescription-drug-coverage/prescriptiondrugcovgenin>

KILL SWITCH: Annual deductible, TrOOP, and premium thresholds. Verify each plan year.

Extra Help / LIS Program (SSA)

<https://www.ssa.gov/medicare/part-d-extra-help>

Screen every client for Low Income Subsidy. KILL SWITCH: Income thresholds change annually.

CMS Medicare Marketing Guidelines (MMG)

<https://www.cms.gov/medicare/health-drug-plans/managed-care-marketing>

Governs all U.S. agent marketing and sales conduct for Part D plans. Required reading.

Inflation Reduction Act — CMS

<https://www.cms.gov/inflation-reduction-act-and-medicare>

IRA changes: \$35 insulin cap, donut hole elimination, catastrophic coverage updates.

YOUR NEXT STEP: AgentRisePro

The Medicare Agent Workspace built by a 25-year U.S. Medicare field veteran — for U.S. agents who are serious about AEP.

AGENT PILOT \$38/month	AGENT PRO \$68/month	AGENCY \$83/month
Solo independent agent AEP launch special	Full features + Voice AI when live	Multi-agent teams White label ready
Script Desk Launch Kit Calendar Basic CRM	Everything in Pilot + GPT Voice (coming) + Advanced CRM + Carrier Tools	Everything in Pro + Team Dashboard + Agency Reporting + Priority Support
Annual: \$368/yr (Save \$88 — 2 mo free)	Annual: \$658/yr (Save \$158 — 2 mo free)	Annual: \$803/yr (Save \$193 — 2 mo free)

- Script Desk — Medicare scripts by product & situation
- Agent Launch Kit — all 5 briefs + 100+ questions
- Fresh Pour — agent community & audio lounge
- Training Calendar — built-in appointment by block
- Book of Business CRM — track your U.S. agents
- GPT Voice AI — hands-free in the car (coming Q1 2027)
- Download & share tools
- U.S. carrier reference library
- CMS compliance reminders built in

■ 7-DAY FREE TRIAL — NO CREDIT CARD REQUIRED

Try the full app free for 7 days. No obligation. No credit card. Cancel anytime. Pilot pricing locked in for early adopters during AEP 2026. Download the Medicare Advantage Launch Blueprint PDF for just \$4.99 — or get it FREE with your 7-day trial.

agentrisepro.com | projectagentrise.netlify.app | 121 Urban Media LLC

This guide is prepared for U.S. Medicare agents and beneficiaries only. Not for consumer distribution without agent supervision. All Medicare rules, premiums, and thresholds are subject to annual change by CMS. Kill Switch items must be verified at medicare.gov and cms.gov before each use. © 121 Urban Media LLC | AgentRisePro | All rights reserved.